

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	730
	A Division of the Merced County Department of Public Health	Effective Date:	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	09/2018
		Review Date:	04/2028

HYPERGLYCEMIA	
- Both DKA and HHNK are characterized by: Thirst, increased urination, drowsiness, confusion, dehydration, nausea, vomiting, and missed insulin doses, or non-compliance with diabetic medications - Patients with DKA may also present with: Fruity odor on breath, deep and rapid respirations (Kussmaul respirations) - Both DKA and HHNK can be life threatening if left untreated - Both DKA and HHNK are associated with possible infections - Patients with hyperglycemia should be screened for potential sepsis	
ADULT	PEDIATRIC
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen - Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations if needed. Blood Sugar Check.	
ALS	
Follow BLS procedures if applicable. Obtain 4-Lead ECG. Capnography – EtCO ₂ readings of ≤ 25 mmHg are suggestive of acidosis (DKA).	
Vascular Access - If blood glucose is > 400 mg/dl, and the patient shows no signs of fluid overload, administer 500 ml bolus - Reassess patient after each bolus for signs of fluid overload	Vascular Access - If blood glucose is > 200 mg/dl, EtCO₂ of ≤ 25 mmHg, and the patient shows no signs of fluid overload - Administer a fluid bolus of 20 mL/kg
	Base Hospital Physician Orders Only
	Make base contact for repeat fluid boluses.

Signatures on File

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